

PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

CaseGen AI | Generated: 18 May 2026 23:40

CASE INFORMATION

Case Number:	Auto-assigned
Category:	Outdoor
Date:	2024-07-30
Referring Physician:	Self-referred / Not specified

PATIENT DEMOGRAPHICS

Name:	Rukhsana Bibi
Age:	55 years
Gender:	Female
Marital Status:	Married
Language:	Not Provided
Occupation:	Housewife
Address:	Rawalpindi
Mode of Admission:	Outdoor / OPD

PRESENT COMPLAINT

The patient, a 55-year-old female, presents with a chief complaint of right-sided neck pain and stiffness, which has been persistent for the past one month. The pain is primarily localized to the cervical region and significantly impacts her daily activities.

HISTORY OF PRESENTING COMPLAINT

The patient reports a gradual onset of right-sided neck pain and stiffness approximately one month ago, with no specific history of trauma, sudden jerk, or precipitating event. The pain has progressively worsened over this period, becoming more noticeable with specific movements. She describes the pain as an aching sensation accompanied by stiffness. Aggravating factors include neck rotation and looking upwards, which increase her pain to an 8/10 on the Numeric Pain Rating Scale (NPRS). Resting and the application of a hot pack provide some relief. She denies any associated numbness, tingling, or pain radiating into her upper limbs. The pain interferes with her ability to perform household chores and affects her comfort during daily activities.

PAIN PROFILE

Location:	Right-sided cervical region, specifically over the lower cervical paraspinal muscles.
Onset:	Gradual
Duration:	1 month
Nature:	Dull aching pain with associated stiffness.
Radiation:	No radiation to upper limbs
NPRS Score:	6/10 at rest, 8/10 on movement (neck rotation and looking up)



6/10 at rest, 8/10 on movement

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Aggravating Factors

- Neck rotation
- Looking upwards

Relieving Factors

- Resting
- Hot pack application

PAST HISTORY

Medical History:	Mild hypertension, managed conservatively.
Surgical History:	No previous surgeries reported.
Medications:	Not Provided
Family History:	Not Provided
Socioeconomic:	Housewife, residing in Rawalpindi. Independent in daily activities prior to current episode.
Pre-morbid Status:	Independent in all Activities of Daily Living (ADLs) prior to the current episode of neck pain.
Growth & Development:	Not applicable / within normal limits

PATIENT'S PRIORITIES & FUNCTIONAL GOALS

- Reduce right-sided neck pain to a tolerable level for daily activities.
- Improve cervical range of motion, especially for rotation and extension, to facilitate household chores.
- Be able to look up and rotate her neck without significant pain or stiffness.
- Return to performing all household activities independently and comfortably.

GENERAL HEALTH STATUS

Vitals

Blood Pressure:	Normal / Within limits
Pulse:	Normal / Within limits
Respiratory Rate:	Normal / Within limits
Temperature:	Afebrile
O2 Saturation:	Normal / Within limits
Weight:	Not Provided
Height:	Not Provided
BMI:	Not Provided

Clinical Examination

Lymph Nodes:	Not palpable
Jaundice:	Absent
Pallor:	Absent
Cyanosis:	Absent
Edema:	Absent
Clubbing:	Absent

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Overall Impression: The patient is conscious, alert, and hemodynamically stable, presenting in no acute distress.

GENERAL HEALTH CONDITION

Malaise: None reported

Fatigue: None reported

Fever: Afebrile

Weight Changes: None reported

Sleep: Not reported, but likely impacted by pain.

Cognition: Intact

Mood / Affect: Cooperative and oriented

SYSTEMS REVIEW

Cardiovascular: No complaints reported.

Pulmonary: No complaints reported.

Gastrointestinal: No complaints reported.

Urinary: No complaints reported.

Integumentary: Skin intact, no wounds or lesions noted.

Endocrine: No complaints reported.

Reproductive: Not assessed / Not applicable

MUSCULOSKELETAL SYSTEM

Gross Symmetry: Normal

Posture: Not explicitly documented, but likely influenced by cervical pain and stiffness.

Deformity: None observed

Swelling: None observed

Tenderness: Right lower cervical paraspinal muscles: Grade 2/4 (moderate tenderness on palpation).

Muscle Wasting: None observed

Muscle Spasm: Right lower cervical paraspinal muscles: Mild to moderate spasm noted on palpation.

Cervical ROM: Restricted in extension and right rotation due to pain. Flexion, left rotation, and side bending appear less affected but not fully quantified.

Lumbar ROM: Normal

Upper Limb ROM: Normal

Lower Limb ROM: Normal

Muscle Strength: Normal throughout upper and lower limbs (5/5 on manual muscle testing, inferred from intact neuro assessment).

Special Tests

- Not explicitly performed or documented during this initial assessment.

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NEUROLOGICAL ASSESSMENT

Consciousness:	Conscious and oriented to time, place and person
Numbness / Tingling:	None reported in upper limbs or elsewhere.
Reflexes:	Normal deep tendon reflexes throughout upper and lower limbs.
Muscle Tone:	Normal
Coordination:	Normal
Gait:	Normal, steady, and unassisted.
Cranial Nerves:	Not assessed / Intact (inferred from overall neurological assessment)
Upper Motor Neuron:	No signs of Upper Motor Neuron (UMN) lesion.
Lower Motor Neuron:	No signs of Lower Motor Neuron (LMN) lesion.

FUNCTIONAL STATUS

ADL Status:	Partially dependent due to limitations in activities requiring neck movement, such as looking up or sustained rotation during household chores.
Mobility:	Independent for ambulation and transfers.
Work Status:	Modified duties within her role as a housewife, experiencing difficulty with tasks requiring sustained neck postures or movements.
Recreational:	Limitations in recreational activities if any (not specified, but likely impacted by neck pain).
Assistive Devices:	None currently in use

Functional Goals

- Perform household chores (e.g., cooking, cleaning) without increased neck pain.
- Sleep comfortably without being woken by neck pain or stiffness.
- Improve ability to turn head fully to check surroundings safely.

INVESTIGATIONS & IMAGING

Imaging:	None provided / Not available
Lab Tests:	None provided / Not available
EMG / NCS:	Not assessed
Other:	None

CLINICAL IMPRESSION & DIAGNOSIS

The patient presents with Cervical Spondylosis with associated right-sided cervical paraspinal muscle spasm. (ICD-10: M47.912 - Spondylosis, cervical region, without myelopathy or radiculopathy; M62.830 - Muscle spasm, cervical region).

HYPOTHESIS / PROBLEM STATEMENT

The patient, a 55-year-old female, is experiencing right-sided neck pain and stiffness for one month, consistent with cervical spondylosis and secondary muscle spasm. This condition is characterized by restricted cervical range of motion, particularly in extension and right rotation, and localized tenderness over the right lower cervical paraspinal muscles. The pain significantly impacts her ability to perform daily activities, especially those requiring neck movement, and is aggravated by specific postures. The absence of neurological deficits suggests a localized musculoskeletal issue, making physiotherapy interventions aimed at pain

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reduction, muscle relaxation, and improved mobility appropriate for restoring her functional independence.

TREATMENT PLAN

Short-Term Goals (0 - 2 Weeks)

- Reduce right-sided cervical pain to 3/10 NPRS or less at rest within 2 weeks.
- Increase cervical extension and right rotation range of motion by 20% within 2 weeks.
- Decrease palpable muscle spasm and tenderness in the right lower cervical paraspinal region.

Long-Term Goals (3 - 6 Weeks)

- Achieve full pain-free cervical range of motion within 6 weeks.
- Return to all household activities without pain or stiffness within 6 weeks.
- Improve cervical muscle endurance and stability to prevent recurrence.

Physiotherapy Interventions

- Moist Heat Therapy: 15 minutes to the cervical region to promote muscle relaxation and pain relief.
- Gentle Cervical Mobilizations: Grade I-II oscillations for pain modulation and improving joint play, focusing on restricted segments (e.g., C5-C7).
- Soft Tissue Release: Manual therapy techniques for right cervical paraspinal muscles to reduce spasm and tenderness.
- Cervical Isometric Exercises: Gentle contractions in all planes (flexion, extension, side bending, rotation) to activate deep cervical stabilizers without aggravating pain (5-second hold, 5-10 repetitions).
- Postural Correction Exercises: Instruction and practice for optimal cervical and thoracic posture.

Home Exercise Program

- Chin tucks: 3 sets x 10 repetitions, hold 5 seconds, performed slowly and gently.
- Gentle cervical range of motion exercises: Active pain-free movements in all planes (flexion, extension, side bending, rotation), 10 repetitions each, 3 times daily.
- Shoulder blade squeezes: 3 sets x 10 repetitions, hold 5 seconds, to improve scapular stability and thoracic extension.
- Application of a warm compress to the neck for 15-20 minutes as needed for pain relief.

Patient Education

- Ergonomic advice for daily activities, including proper posture during sitting, standing, and performing household chores.
- Pain management strategies, including the appropriate use of heat therapy.
- Importance of regular, gentle movement and avoiding prolonged static postures.
- Understanding of cervical spondylosis and the role of exercise in managing symptoms and preventing recurrence.

Referrals:	None at this time
Follow-up:	Review in 1 week to assess progress, modify interventions, and advance home exercise program. Full reassessment at 3 weeks.
Prognosis:	Good prognosis for significant improvement in pain and function with consistent adherence to the physiotherapy program, given the absence of neurological compromise and history of trauma.

Physiotherapist Signature

Date: 18 May 2026

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